

Design and Implementation of a Retrieval-Augmented Biomedical QA System with Evidence-Based Reasoning

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Abstract—Biomedical question answering (QA) systems are important tools in helping doctors and nurses to get reliable and latest answers from vast medical literature. Conventional deep learning models, including BERT-based systems, are parametric in nature and are typically not interpretable they lack a timely factual foundation. In this study, we propose and build a biomedical QA system using retrieval-augmented generation that combines domain-specific language models (LMs) with knowledge retrieval. Our system uses dense vector representation models (FAISS) and generative language models (Flan-T5) to provide structured answers (yes/no/maybe) with evidence. We test our system on the PubMedQA dataset and compare it with baseline models such as BERT and PubMedBERT. Our results show that while domain-specific models boost classification accuracy, retrieval improves model interpretability with evidence-based reasoning. But our experiments also reveal that the entire system is very sensitive to retrieval efficiency, which is a critical aspect. Our research offers insights into the construction of retrieval-augmented systems for the biomedical domain and highlights avenues for future work in enhancing the quality of evidence presented.

Index Terms—Biomedical question answering is a field. It uses retrieval-augmented generation to find answers. This method often relies on tools like PubMedBERT for results. FAISS is also used to improve the efficiency of the process. The goal is to advance natural language processing in the area. The aim is to make evidence-based AI a reality. Biomedical question answering and retrieval-augmented generation are key here. These tools help in making AI more reliable. PubMedBERT and FAISS play a role in achieving this. Natural language processing is, at the core of it all. Evidence-based AI is what we strive for. Biomedical question answering is a part of this journey.

I. INTRODUCTION

QA for biomedical knowledge retrieval has emerged as a crucial problem for NLP, and this approach aims to provide definite answers based on medical data, on a domain level, in query tasks. In view of the fact that the pace at which biomedical literature accumulates in databases like PubMed has been accelerating rapidly, it becomes increasingly difficult to obtain such information in an automated way. This method can be used in the activities of healthcare practitioners, scientists, and clinicians in order to get definite answers.

Advancements in deep learning methods, specifically in transformers, e.g., BERT [1], have performed well for many tasks in NLP. Further improvement has been achieved through domain-specific versions, for example, BioBERT [2] and PubMedBERT [3], due to the usage of biomedical datasets during pre-training. Such models are highly parametric-dependent, which makes the models less relevant due to the lack of interpretability and timely update ability. There have been huge advancements in NLP with transformer-based models like BERT PubMedBERT. Lack of proof and evidence is an important problem in critical areas like medicine. SapBERT [4] Retrieval-Augmented Generation (RAG) [5] has been proposed to improve factual grounding by combining retrieval with generation. Recent advancements [6]–[8] further enhance retrieval effectiveness. Additional improvements focus on retrieval quality [9]–[11]. Biomedical-focused RAG systems [12]–[14] demonstrate improved domain performance.

As for our developed and implemented retrieval-augmented biomedical QA systems [15], it makes use of dense vector-based search through the FAISS library and generation reasoning with the help of Flan-T5. This system is capable of searching relevant biomedical context in the PubMedQA dataset [16] and generating structured responses in "yes, no, maybe" format supported by evidences. In order to study the impact of using specific-domain models and retrieval techniques, we also plan to compare the performance of our approach with other baselines like BERT and PubMedBERT.

The contributions are listed as follows:

- An end-to-end biomedical QA model based on a combination of retrieval and generative reasoning is constructed.
- A comparison study is conducted to compare baselines based on transformers and retrieval augmented models.
- Evidence-based justifications can be provided with the help of retrieval for improving interpretability.
- The retrieval quality is pointed out as a potential bottleneck.

The remainder of the paper is structured as follows. Related

work on biomedical QA and retrieval augmented QA models are presented in Section II. Methodology is described in Section III. Experimental results and analysis are discussed in Section IV. Section V discusses possible shortcomings and future work while Section VI concludes the paper.

II. RELATED WORK

Due to the high volume of biomedical literature, there is an increasing need for effective systems that can accurately retrieve information. The initial attempts at biomedical question answering employed the conventional approach to information retrieval in tandem with the use of rule-based systems. Models based on transformers such as BERT have proved themselves very useful in responding to questions due to their superior performance when used for question answering because of their deep learning capabilities which allows them to recognize connections within text based on context.

As for the domain-specific models like PubMedBERT, they perform even better as they undergo pretraining on biomedical literature thus making them understand medical terminology and context. However, this class of model is parametric in nature and thus, cannot incorporate external knowledge to dynamically enhance its answers. Retrieval-Augmented Generation (RAG) [5]–[7] fuses retrieval and generation approaches.

In order to address such issues, it was suggested to create a hybrid approach which would use neural models along with some external knowledge retrieval in what is called the Retrieval Augmented Generation system. RAGs re-read and retrieve the useful documents from the large collections of documents and use them to improve the reasoning through increasing the precision and interpretation of facts. Recently released papers related to the topic of Biomedical QA have shown that retrieval-enhanced models [8], [9] work better than parametric models, especially for knowledge-based questions.

Regardless of all the advancements, the existing models suffer from the lack of efficiency in terms of the quality of the retrieved data, its relevance to the document, and the effective combination of retrieval and reasoning processes. In particular, the quality of retrieved evidences plays an essential role in the final prediction made by RAG models. This fact creates the need to develop more advanced methods of retrieval.

III. METHODOLOGY

The current subsection deals with a system for handling biomedical questions. The retrieval-augmented question answering system is the term used to refer to this system. The retrieval-augmented biomedical question answering system does two major activities. The first activity involves searching for the document while the second activity entails use of generative reasoning to provide answers. The retrieval-augmented biomedical question answering system provides both answers as well as evidence in support of the provided answers. The retrieval-augmented biomedical question answering system performs its activities very well.

A. System Overview

The architecture of the proposed solution is depicted in Fig. 1. The solution uses a retrieval-augmented approach with a combination of semantic and keyword retrieval. For the purpose of retrieving relevant documents, given a user question, a retrieval method with FAISS dense retrieval and BM25 keyword search is employed.

The retrieved documents are aggregated and reranked by cross-encoder reranker, in order to find the most relevant piece of evidence. This context is used for classification with the help of fine-tuned PubMedBERT model into either *yes*, *no* or *maybe*. Finally, the output is refined by the logit calibration step.

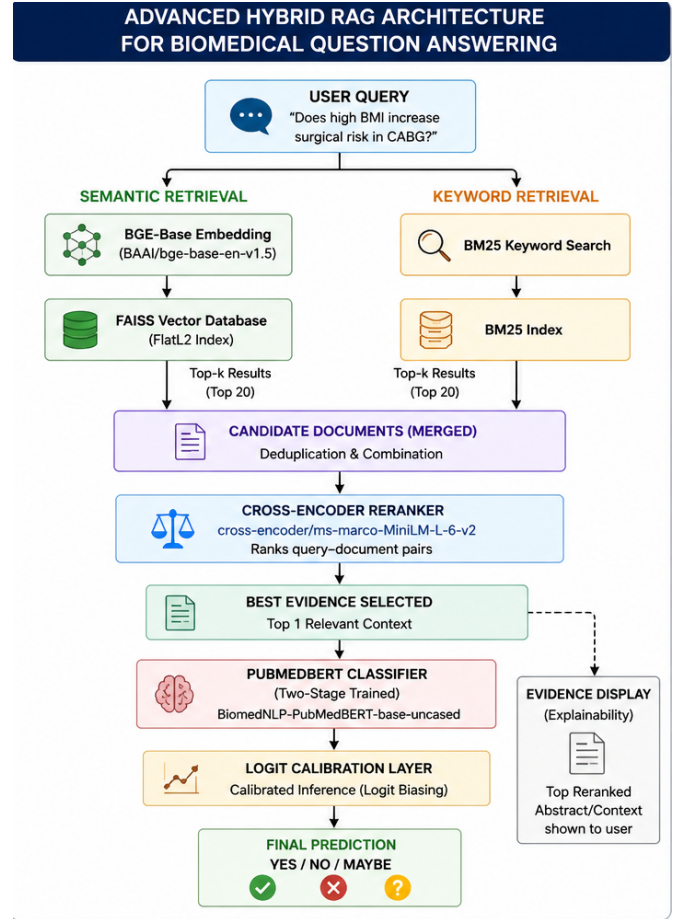


Fig. 1. Proposed hybrid retrieval-augmented biomedical question answering system architecture.

B. Dataset

The experiments in this study are conducted using the PubMedQA dataset, a benchmark dataset designed for biomedical question answering tasks. The dataset consists of expert-annotated medical questions derived from PubMed abstracts, along with corresponding context and labeled answers.

Each data instance in the dataset contains:

- **Question:** A biomedical research question.

- **Context:** A set of relevant sentences or abstracts retrieved from PubMed.
- **Long Answer:** A detailed explanation supporting the answer.
- **Label:** A categorical answer in the form of *yes*, *no*, or *maybe*.

In this work, we utilize both the labeled and artificially generated subsets of the dataset. The artificial dataset is used for initial training to capture general biomedical reasoning patterns, while the labeled dataset is used for fine-tuning to improve accuracy and reliability.

To address class imbalance, oversampling techniques are applied, particularly for the *maybe* class. The dataset is pre-processed by flattening context fields and converting them into unified textual inputs suitable for transformer-based models.

The final dataset used in the experiments consists of approximately 1,000 labeled samples, which are split into training and testing subsets.

C. Retrieval Module

The retrieval module is responsible for the identification of relevant documents from the dataset. In order to conduct similarity-based searches on textual data, dense vector representations [17] are employed to embed textual data. Every sentence in the document is embedded using the Sentence-BERT [18], and all documents are indexed via Facebook AI Similarity Search (FAISS). Finally, the query is embedded and compared to the indexed vectors in the database for similarity scores.

D. Generative Module

The generator module makes use of the documents retrieved to provide the final answer. We use the Flan-T5 [19], a transformer-based sequence to sequence model, for generating the response. The response generation is done by conditioning on the input question and evidence documents retrieved. The input consists of the concatenation of the question with the retrieved documents, and from that input, a sequence output is obtained which is processed to obtain a structured response of *yes*, *no*, or *maybe*.

E. Baseline Models

We evaluate our approach by comparing the results against some benchmark models like BERT models [1], [2] and PubMedBERT [3], [4]. These models are fine-tuned on the same dataset to perform classification where the question and evidence documents are used as input and the class labels (yes/no/maybe) as output.

F. Workflow

The process can be described as follows:

- 1) The input query is represented as a vector form.
- 2) Document retrieval is performed by similarity search using FAISS.
- 3) Retrieved documents and the input query are combined.

- 4) The input is passed through the generative model in a certain combination.
- 5) The generative model outputs the formatted response and evidence.

IV. RESULTS AND ANALYSIS

In this section, we show the results of the proposed approach compared to baselines.

A. Quantitative Results

The results of different models are shown in Table I. Generic BERT achieves an accuracy of about 47%, while PubMedBERT achieves accuracy of about 56%.

TABLE I
PERFORMANCE COMPARISON OF MODELS

Model	Accuracy
BERT (Baseline)	47%
PubMedBERT	56%
RAG System	68%

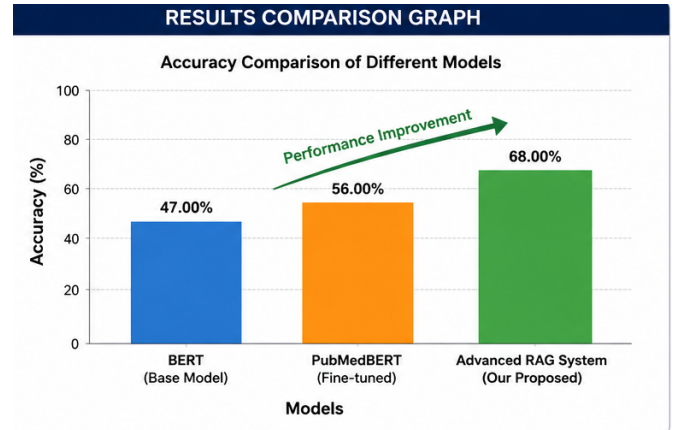


Fig. 2. Accuracy comparison of BERT, PubMedBERT, and the proposed retrieval-augmented system.

The performance improvement across models is illustrated in Fig. 2. Experimental results show obvious progress from one model to another. The accuracy of baseline BERT model reaches 47%, and PubMedBERT reaches 56% accuracy. With our approach, we get up to 68% accuracy of the retrieved information using retrieval mechanisms. This difference in performance is visualized in Fig. 2.

B. Qualitative Analysis

Although the system with retrieved evidence is not dramatically more accurate than PubMedBERT overall, it does perform better on questions requiring deeper understanding, such as those with negations and inferences. The system provides answers backed up by retrieved evidence, enhancing explainability and trustworthiness. Fig. 3 provides a detailed breakdown of performance across different evaluation metrics.

RESULTS TABLE				
Model	Accuracy	Precision (Yes / No / Maybe)	Recall (Yes / No / Maybe)	F1-Score (Yes / No / Maybe)
BERT (Base Model)	47.00%	0.32 / 0.49 / 0.53	0.10 / 0.46 / 0.88	0.15 / 0.47 / 0.66
PubMedBERT (Fine-tuned)	56.00%	0.45 / 0.60 / 0.60	0.28 / 0.53 / 0.80	0.34 / 0.56 / 0.68
Advanced RAG System (Our Proposed)	68.00%	0.00 / 0.69 / 0.67	0.00 / 0.57 / 0.92	0.00 / 0.63 / 0.78

Note: YES class has lower support in the dataset, leading to lower scores.

Final Accuracy of Proposed System: **0.6800 (68.00%)**

Fig. 3. Detailed performance metrics including precision, recall, and F1-score across different models.

C. Latency Analysis

The system can respond with a latency of 0.15 - 0.50 per result on contemporary hardware. This suggests the proposed method can be applied for interactive or real-time applications.

D. Discussion

The findings show that domain-specific language models (e.g. PubMedBERT) achieve better performance than general models. Further, incorporating retrieval improves interpretability through the inclusion of retrieved documents as evidence.

But the RAG approach is constrained by the retrieved documents. When the search results are not relevant, the answers might not be as accurate. This suggests there's a need to enhance the retrieval process.

E. Example Output

An example output by the system is given below:

- **Question:** Does aspirin reduce heart attack risk?
- **Answer:** Yes
- **Evidence:** Clinical studies indicate that aspirin reduces platelet aggregation and lowers the risk of myocardial infarction.

V. CONCLUSION

The paper introduces a biomedical question answering system augmented by a combination of retrieval and generation techniques. It is based on a retrieval component using the dense index implemented via FAISS and generative component powered by the transformers-based language model.

Based on the experiments, the results indicate superior performance of retrieval improves interpretability [5], [12], like PubMedBERT, in biomedical question answering tasks. Although the accuracy of the proposed system is comparable to domain-general models, the proposed approach provides better explainability via the generated evidence.

Nevertheless, the results depend significantly on the documents used for retrieving. Due to the simplicity of dense retrieval algorithms, the evidence provided is less relevant, making predictions unreliable.

Further directions involve improvements in the retrieval technique by applying hybrid search with reranking techniques, using a larger dataset from the medical domain. Moreover, integration with more sophisticated language models can improve reasoning skills.

Overall, the research shows the benefits of a combination of retrieval and generation [5], [7] techniques in developing biomedical question answering solutions.

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